جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : امل ناجي بديع غبريال

المحافظة : القاهره **الحي / المدينة :** المرج **العنوان :** 6 ش خالد الغزازي - مدينه الصفا

رقم البطاقة : 27306222400361 **رقم التليفون :** 01274999136 **تاريخ تسجيل الإستمارة :** 20-10-2026

العمر(تاريخ الميلاد) : 22-06-1973 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي
التوقيع:(----------)

المقر بما فيه
التوقيع:(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: امل ناجي بديع غبريال
National ID: 27306222400361

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Supportive and palliative Treatment**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين



National ID – Front
[[DOCTYPE:ID_FRONT]]



[[DOCTYPEID_BACK]]

جوستاف روسي

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

08/19/2026

MDT FORM

Name:	Nagy Badia Ghbrial	MRN:	12760
Age:	53 Y	ID:	27306222400361
Menopausal Sta	POST	Phone:*	01274999136
NRS SCORE:	PS 1	Marital Status:	MARRIED
BMI:		Family History:	-VE
Gender			
CO Morbidity:	NAD	Reason Of MDT:	NEW CASE
Surgical History:*	CS, CHOLECYSTECTOMY	ECOG Performance Status	○ 0 ◉ 1 ○ 2 ○ 3 ○ 4

Clinical Data:

BX: IDC G II

ER: 6/8 **PR:** 5/8 **HER2:** +2 AMP **KI67:** 45

History:

Sonomammography:
ACR C
RT IRREGULAR MASS 8*9 CLOCK,
3 OTHER ADJACENT LESIONS,
THICK SKIN AROUND NAC
ENLARGED INF AX LN
T4B N1 M0

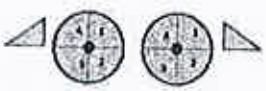
Biomarker/Molecular

Mets Workup:
CT CAP NAD
BONE SCAN FREE

Recommendations:
FOR CESM , THEN NACT AND THEN POSSIPLE RT MRM

MDT Outcome & Plan
For CESM review at surgery Clinic
Portacath and ECHO
start NACT TCHP 6cycles then for Rt MRM

R L



TNM

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:
T4B N1 M0

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY

GUSTAVE ROUSSY INTERNATIONAL EGYPT

Gustave Roussy International Egypt - Pathology Lab
جوستاف روسي انترناشونال - مصر معمل الباثولوجي

جوستاف روسي

Patient Name	امل ناجى بديع	**Case No.**	341808
Sex/Age	Female 53 Y	**Patient No.**	24727
Registered Date	13/7/2026 01:21:57 PM	**Referred By Dr.**	Interventional Radiology
Validation Date	27/7/2026 02:36:55 PM	**Branch**	Gustave Roussy
Reporting Date	27/7/2026 02:41:40 PM	**Pathology No.**	2198/26

Clinical Data: Right breast mass (BIRADS 5).

Nature of Specimen: Tru-cut biopsy, collected on 13/07/2026. Cold ischemia time is 20 seconds.

Gross Examination:
-Three fragmented grey, white tissue cores, ranging in size between 0.5 cm & 1 cm long, totally submitted.

Technique:
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:
-**Histologic type:** Invasive duct carcinoma.
- **Histologic grade:** 2 (differentiation 3, pleomorphism 2, mitosis 1).
- **DCIS:** Absent.
- **Microcalcification:** Absent.
- **Angioinvasion:** Absent.
- **Perineural invasion:** Absent.
- **Tumor infiltrating lymphocytes (TILs):** Mild.

Results:
* **IPT** (All controls showed appropriate reactivity):
- **ER:** Strong nuclear reaction in 30% of tumor cells (score 6/8).
- **PR:** Strong nuclear reaction in 10% of tumor cells (score 5/8).
- **Her2:** Moderate complete membrane reaction in more than 10% of tumor cells (score 2+).
- **SISH:** Amplified (Average Her2 copy No.= (5.85), Her2/CEP17 ratio = (3.6)).
- **Ki67%:** 45%.

Conclusion:
Right Breast Mass; Tru-Cut Biopsy:
- Invasive duct carcinoma, Grade 2.
IPT:
- **ER:** Positive (score 6/8).
- **PR:** Positive (score 5/8).
- **Her2:** Equivocal (score 2+).

CAP

Mammogram
[[DOCTYPE:MAMMOGRAM]]

GUSTAVE ROUSSY INTERNATIONAL
Gustave Roussy International Egypt- Pathology Lab
جوستاف روسي انترناشونال – مصر معمل الباثولوجي
جوستاف روسي

Patient Name	امل ناجى بديع	**Case No.**	**341808**
Sex/Age	**Female 53 Y**	**Patient No.**	**24727**
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- **SISH**: Amplified
- **Ki67%**: 45%.

***Scoring system: "ER, PR"*
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%
and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
*** Scoring system: "Her2"*
Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahim Ghonaim, MD.

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Patient's Name:	**Amal Nagy Badi' Ghibrial**
Date:	**2026-08-05**
Referred by:	**Prof.Dr. no doctor**
Hospital:	**Goustave Roussey International Egypt**

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of breast CA/

I- CHEST:

- Right breast lesion measuring 33 x 23 mm with skin thickening for sono-mammography.
- Right axillary lymph nodes largest measuring 23 x 15 mm.
- Tiny right upper lobe calcified pulmonary nodule likely post inflammatory/ post granulomatous sequelae.
- No pulmonary masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.
- Bilateral elastofibroma dorsi.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Enlarged spleen showing no focal lesions.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- IUCD is seen within the uterus.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- D8 and L3 vertebral bodies lytic foci.

Opinion:

- **Right breast lesion with right axillary lymph nodes for sono-mammography.**
- **D8 and L3 vertebeal bodies lytic foci for bone scan.**

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient's Name:	**Amal Nagy Badi' Ghibrial**
Date:	**2026-08-05**
Referred by:	**Prof.Dr. no doctor**
Hospital:	**Goustave Roussey International Egypt**

❖ **No pulmonary or hepatic metastatic lesions.**
Dr.sherif Wagdy

Dr.sherif Wagdy

CT Scan
[[DOCTYPE:CT_SCAN]]

Name of Patient :
Referred by prof. Dr :
Date : / / 20

Name: Amal nagy
Date: 9/7/2026

SM H
مستشفى السيدة العذراء بالزيتون

SONOMAMMOGRAPHY OF BOTH BREASTS

Indication: right breast lump

Findings:

Mammogram:

- Both breasts display normal skin thickness with normal appearance of the nipples and subareolar regions.
- The breast parenchyma shows a heterogeneously dense fibroglandular pattern (ACR breast density c).
- Right UOQ irregular densities.

Ultrasound:

- Right breast irregular speculated margin hypoechoic mass seen at 8-9 o'clock position, 4 cm from nipple measuring 33x25 mm. 3 other adjacent similar lesions are seen 9-11 o'clock position, 4-6 cm from nipple measuring 12.5x7 mm, 17x13.5 mm and 16x12 mm.

- Enlarged infiltrated right axillary lymph nodes with globular shape and eccentric hilum. The largest measures 14 mm with 7 mm cortical thickness.

Left breast: No suspicious solid or cystic masses or abnormal calcifications. No suspicious left axillary lymphadenopathy

Impression:

Right breast multifocal disease with infiltrated right axillary lymph nodes for histopathology (BIRADS 5)

Much obliged

Dr/Nermeen Nasry,MD

A. Professor of Radiology, Ain Shams University

BIRADS: THE BREAST IMAGING AND REPORTING DATA SYSTEM	
BIRADS 0	Incomplete. Needs additional imaging evaluation.
BIRADS 1	Negative. No lesion found. Routine follow up for age
BIRADS 2	Benign findings. No malignant features
BIRADS 3	Probably benign finding. Malignancy is highly unlikely, initial short interval follow up
BIRADS 4	Suspicious abnormality. Low to moderate probability of cancer, biopsy should be considered.
BIRADS 5	Highly suggestive of malignancy, requires tissue sampling
BIRADS 6	Known cancer. Biopsy proven malignancy, prior to institution of therapy

١٢٠ شارع طومانباى - الزيتون - القاهرة
ت: ٢٢٥٩١٩٩٩ - ٢٤٥٢٩٦٣١
موبايل: ٠١٢١١١٠٩٥٥١

Mammogram
[[DOCTYPE:MAMMOGRAM]]

NILE SCAN & LABS

مركز النيل للأشعة و التحاليل

Name : Amal Nagy Badi' Ghibrial	Date : 13-08-2026 07:49
Branch : Dokki	Medical No. : 438581
Ref.Physician Prof : Goustav Roussey Egypt	

NILE DIAGNOSTIC CENTER

NUCLEAR MEDICINE BONE SCAN REPORT

CLINICAL INDICATION: 53-year-old female with newly diagnosed left breast carcinoma. No prior surgery, chemotherapy, or radiotherapy.
Referred for baseline skeletal staging.

PROCEDURE: Whole-body planar bone scintigraphy, anterior and posterior views, 3 hours following intravenous administration of 99mTc-MDP.

FINDINGS:

- Axial and appendicular skeleton shows homogeneous symmetrical tracer distribution.
- No focal or multifocal areas of increased osteoblastic activity are identified.
- Mild bilateral knee and tarsal joint uptake—degenerative, per history.
- Renal excretion is prompt and symmetric.
- No abnormal soft tissue uptake.

IMPRESSION:

- No scintigraphic evidence of osseous metastases.
- Follow up as per clinical pathway.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

Bone Scan
[[DOCTYPE:BONE_SCAN]]

USTAVE
OUSSY
RNATIONAL

جوستاف روسي انترناشونال

ECHO REPORT

NAME : امل ناجي بديع DATE: 24/9/2024

D NUMBER : / SEX : ♀ AGE: 53 Y.

:	69.1.	55-75%	PWT	0.7	0.6-1.1cm
/EDD	4.8	3.5-5.8cm	AO.R	2.7	3-3.7cm
/ESD		2-4cm	LA	3.6	2-4cm
;		25-45%	RV		0.7-2.3com(supine)
NT	0.8	0.6-1.1cm	AO.CUSP		1.5-2.6cm

-V Dimensions ——— oimal

-V Wall Thickness ——— oimal

-V Diastolic Function DD grade I

;WMA ——— سليم

-A and Ao Root Dimensions ——— oimal

Mitral Valve ——— oimal

Aortic Valve ——— oimal

Tricuspid Valve trivial TR with normal SPAP

Pulmonary Valve ——— oimal

Right Side and Systolic Function ——— oimal

ntracardiac Masses and Effusion ———

DR.

Echo
[[DOCTYPE:ECHO]]